

Question:

A 34-year-old woman presents to her OB/GYN with complaints of missing her last 3 periods as well as intermittent spontaneous milky-white nipple discharge bilaterally for the past 3 months. Vital signs are stable and within normal limits. Neurologic examination is without abnormality, including normal visual fields. Serology and MRI of the brain are ordered, with results pending. Which of the following sets of laboratory results would be expected in this patient?

Answer Choices:

- A. Decreased prolactin, decreased FSH, decreased LH
- B. Decreased prolactin, increased FSH, increased LH
- C. Increased prolactin, decreased FSH, increased LH
- D. Increased prolactin, decreased FSH, decreased LH

Reasoning + Answer:

<think> The patient is experiencing amenorrhea (missed periods) and galactorrhea (milky nipple discharge), which are classic symptoms of hyperprolactinemia. Hyperprolactinemia can be caused by a pituitary adenoma that secretes prolactin. In this case, the prolactin levels are likely increased. Hyperprolactinemia suppresses the release of gonadotropin-releasing hormone (GnRH), which in turn suppresses the release of luteinizing hormone (LH) and follicle-stimulating hormone (FSH) from the anterior pituitary. This leads to decreased levels of LH and FSH, resulting in decreased ovarian function and amenorrhea.

</think>

<answer> D. Increased prolactin, decreased FSH, decreased LH </answer>

